PLANNING BRIEF

Long COVID burden and pilot planning brief

Deterministic synthesis of reviewed national burden, analyst-supplied planning assumptions, explicitly selected implementation context, evidence gaps, and interpretation guardrails.

Brief provenance
National reviewed artifact: meps-2024-long-covid-candidate-20260926T011343Z
Generated: 2026-10-16T00:28:35.290Z

Planning insights summary

Deterministic planning preview — no live AI inference. Selected highlights; full reviewed findings, assumptions, provenance and selected requests follow in the Brief.

Reviewed national evidence

Mean annual all-cause medical spending per person: $14,238
USD/person/year · 95% CI $10,668–$17,807
Reviewed record: meps24-gross-all-cause-spending-mean · Source: meps-hc256-2024

Annualized health care paperwork / navigation time: 24.5 hours/person/year
hours/person/year · 95% CI 12.8–36.2 hours/person/year
Reviewed record: meps24-healthcare-paperwork-time-mean · Source: meps-hc256-2024

Mean K6 nonspecific psychological distress score: 5.9 score (0-24)
score (0-24) · 95% CI 5–6.8 score (0-24)
Reviewed record: meps24-k6-score-mean · Source: meps-hc256-2024

Universe: population definitions, sampling and eligibility restrictions vary by estimate. Denominator: use the matching reviewed record IDs above to inspect the complete denominator and exact source locator in the validated national artifact and Evidence view. This is a compact synopsis; the reviewed Brief below remains authoritative.

One finding per available domain; descriptive highlights, not a ranking of burden.

Selected local implementation context

Reviewed West Virginia context is available. Select only the indicators relevant to your implementation decision.

Analyst assumptions and deterministic planning outputs

Recognition → referral · Navigation support · illustrative-navigation-pilot-default · planning-assumptions-2026-09-v1

Potentially eligible (assumption)	**12,000**	Intended reach (assumption)	**25% (0.25)**
Program capacity (assumption)	**3,000**	Baseline recognition window (assumption)	**12 months**
Planning recognition window (assumption)	**6 months**	Planning horizon (assumption)	**12 months**
Fixed program cost (assumption)	**$300,000**	Cost per participant (assumption)	**$300**
Participants within capacity (result)	**3,000**	Modeled capacity gap (result)	**0**
Person-month exposure difference (result)	**18,000**	Program-resource requirement (result)	**$1,200,000**

Copied from the applied Pilot result. These are assumption-based planning quantities, not forecasts, causal effects, or savings.

Interpretation boundaries

- Gross burden is not attributable burden.
- Local context informs implementation and does not modify national burden.
- Scenario outputs are deterministic planning quantities, not savings, ROI, causal benefit, or observed improvement.
- Out-of-pocket/self-family-paid spending is nested within total spending and must not be added to it.
- K6 measures nonspecific psychological distress and PHQ-2 is a depressive-symptom screen; neither is a psychiatric diagnosis.
- ESAQ time measures are AHRQ annualized hours-per-year measures; later ESAQ responses do not establish continued current Long COVID after the earlier Round 3/1 status anchor.

Decision state

Burden estimates	Reviewed national	**Scenario arithmetic**	Planning assumptions
Evidence parameter	No approved record loaded	**Local context**	Available, not selected
Data requests	None selected		

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Reviewed national burden findings

Economic / healthcare burden

13 reviewed

Measure	Estimate	Confidence interval
Represented population with operational current Long COVID	6.8M	95% CI 5.9M–7.7M
Gross annual all-cause medical spending	$97.3B	95% CI $72.0B–$122.5B
Mean annual all-cause medical spending per person	**Mean:** $14,238 **Typical Range (25th–75th percentile):** $1,116–$14,840 **Median:** $3,624	95% CI $10,668–$17,807
Out-of-pocket annual spending component	$13.5B	95% CI $6.5B–$20.5B
Mean out-of-pocket annual spending per person	$1,980	95% CI $945–$3,014
Mean annual office-based provider visits	12.4 visits/person/year	95% CI 9.4–15.4 visits/person/year
Proportion with at least one office-based provider visit	81.7%	95% CI 77.2%–86.1%

Measure	Estimate	Confidence interval
Mean annual outpatient department provider visits	2.3 visits/person/year	95% CI 1.5–3.1 visits/person/year
Proportion with at least one outpatient department provider visit	33.3%	95% CI 27.9%–38.6%
Mean annual emergency-room visits	0.4 visits/person/year	95% CI 0.3–0.5 visits/person/year
Proportion with at least one emergency-room visit	24.4%	95% CI 19.7%–29%
Mean annual hospital discharges	0.1 discharges/person/year	95% CI 0.1–0.2 discharges/person/year
Proportion with at least one hospital discharge	8.3%	95% CI 5.5%–11.1%

▼ Interpretation and technical details

Domain limitations: All spending and utilization estimates are gross/all-cause descriptive burden. The cohort is classified at the Round 3/1 2024 interview; annual 2024 spending/utilization may occur before or after that point and does not imply year-long symptom persistence. Out-of-pocket spending is nested within total spending and is not additive.

- **Represented population with operational current Long COVID:** Survey-weighted number of status-classifiable U.S. civilian noninstitutionalized adults age 18-64 meeting the documented operational Round 3/1 2024 current-Long-COVID rule.

  System boundary: National descriptive population estimate
- **Gross annual all-cause medical spending:** Calendar-year 2024 gross all-cause health care spending among adults age 18-64 who met the operational current-Long-COVID definition at their Round 3/1 interview in 2024.

  System boundary: National all-cause health care expenditures
- **Mean annual all-cause medical spending per person:** Mean calendar-year 2024 all-cause health care spending among adults age 18-64 who met the operational current-Long-COVID definition at their Round 3/1 interview in 2024.

  System boundary: National all-cause health care expenditures
- **Out-of-pocket annual spending component:** Self/family-paid component of calendar-year 2024 all-cause spending among adults classified operational-current at the Round 3/1 2024 interview.

  System boundary: National all-cause health care expenditures; source-of-payment component
- **Mean out-of-pocket annual spending per person:** Mean self/family-paid component of calendar-year 2024 spending among adults classified operational-current at the Round 3/1 2024 interview.

  System boundary: National all-cause health care expenditures; source-of-payment component

- **Mean annual office-based provider visits:** Mean calendar-year 2024 all-cause office-based provider visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one office-based provider visit:** Share with at least one office-based provider visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual outpatient department provider visits:** Mean calendar-year 2024 all-cause outpatient department provider visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one outpatient department provider visit:** Share with at least one outpatient department provider visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual emergency-room visits:** Mean calendar-year 2024 all-cause emergency-room visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one emergency-room visit:** Share with at least one emergency-room visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual hospital discharges:** Mean calendar-year 2024 all-cause hospital discharges among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one hospital discharge:** Share with at least one hospital discharge during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization

Healthcare-work / administrative burden

4 reviewed

Measure	Estimate	Confidence interval
Annualized time on own medical visits / filling prescriptions	76.5 hours/person/year	95% CI 45.7–107.2 hours/person/year
Annualized time managing medical bills / insurance	27.5 hours/person/year	95% CI 13.5–41.5 hours/person/year
Annualized health care paperwork / navigation time	**Mean:** 24.5 hours/person/year **Typical Range (25th–75th percentile):** 0 hours/person/year–12 hours/person/year **Median:** 3 hours/person/year	95% CI 12.8–36.2 hours/person/year
Prior approval denied or delayed — yes/no response universe	34.3%	95% CI 27.2%–41.5%

▼ **Interpretation and technical details**

Domain limitations: ESAQ native time domains are reported separately and are not summed. ESAQ respondents were classified operational-current at the earlier Round 3/1 2024 anchor; later ESAQ responses do not prove continued current status. ESAQ time measures are top-coded by AHRQ and have item-response limitations.

- **Annualized time on own medical visits / filling prescriptions:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for own medical visits and filling prescriptions among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain
- **Annualized time managing medical bills / insurance:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for paying or managing medical bills and insurance among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain
- **Annualized health care paperwork / navigation time:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for paperwork, finding doctors, plan information, and approvals among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain

- **Prior approval denied or delayed — yes/no response universe:** Proportion reporting that health insurance denied or delayed prior approval among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 previously classified operational-current at the earlier Round 3/1 2024 interview who supplied a substantive Yes/No response.

  System boundary: National ESAQ prior-approval experience

Mental-health / human burden

4 reviewed

Measure	Estimate	Confidence interval
Mean K6 nonspecific psychological distress score	5.9 score (0-24)	95% CI 5–6.8 score (0-24)
Serious nonspecific psychological distress (K6 >=13)	14.1%	95% CI 9.3%–18.8%
Mean PHQ-2 depressive-symptom screen score	1.3 score (0-6)	95% CI 1.1–1.6 score (0-6)
PHQ-2 screen-positive (score >=3)	20.6%	95% CI 14.7%–26.4%

▼ **Interpretation and technical details**

Domain limitations: K6 measures nonspecific psychological distress; PHQ-2 is a depressive-symptom screen. PSAQ respondents were classified operational-current at the earlier Round 3/1 2024 anchor; later PSAQ outcomes do not prove continued current status. Neither is a psychiatric diagnosis, monetized burden, or causal Long COVID effect.

- **Mean K6 nonspecific psychological distress score:** Mean official K6 nonspecific psychological distress score among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ nonspecific psychological distress
- **Serious nonspecific psychological distress (K6 >=13):** Proportion meeting AHRQ's published K6 >=13 serious-distress category among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ nonspecific psychological distress
- **Mean PHQ-2 depressive-symptom screen score:** Mean official PHQ-2 depressive-symptom screening score among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ depressive-symptom screening
- **PHQ-2 screen-positive (score >=3):** Proportion with PHQ-2 score >=3 among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ depressive-symptom screening

Pilot planning assumptions

Illustrative planning defaults — edit before use These inputs are planning assumptions, not observed evidence.

Potentially eligible	**12,000**	Intended reach	**25% (0.25)**
Program capacity	**3,000**	Baseline recognition window	**12 months**
Planning recognition window	**6 months**	Planning horizon	**12 months**
Fixed program cost	**$300,000**	Cost per participant	**$300**

MODELED PATHWAY STEP
Recognition → referral

ASSUMED MECHANISM
Navigation support

Reach, capacity, recognition-window assumptions, and program-resource inputs apply only within this modeled step and mechanism.

Outside this model: Reviewed national burden estimates; Unrelated care-pathway stages; Treatment effectiveness; Clinical outcomes.

Scenario outputs

Deterministic planning quantities from the applied assumptions above; these are not a forecast or observed evidence.

Participants within capacity	**3,000**	Person-month planning exposure difference	**18,000**
Program-resource requirement	**$1,200,000**		

Decision trace

Evidence → explicit analyst context → planning scope → evidence/data gap. This is a provenance and decision ledger, not a proven causal chain.

01 **Released observed burden**
Reviewed national MEPS burden · 21 reviewed estimates · national descriptive evidence

02 **Explicitly selected reviewed local constraint/context**
No explicit local constraint selected

03 **Target pathway step**
Recognition → referral

04 **Planning mechanism**
Navigation support

05 **Measurement / data request needed**
No current operational data request attached

06 **Supported claim boundary**
No reviewed intervention-effect parameter is available. The scenario remains assumption-based; causal benefit, savings, ROI, and preventable-cost claims remain unavailable.

Local implementation context

Reviewed West Virginia state context is available, but no local context has been selected for this brief or used in Pilot planning.

Review the available state evidence on the Local Context page. A record enters this Brief only through an explicit Include-in-Brief selection or an explicit Use-in-Pilot selection.

Intervention-effect evidence

No reviewed intervention-effect parameter is loaded.

Causal benefit, savings, ROI, and preventable-cost claims remain unavailable.

What to collect next

No data request has been created or loaded.

Data requests are planning artifacts, not observed evidence. Add or select them explicitly before export.